

CARESYNC HOSPITAL

Admission & Discharge Standard Operating Procedure (SOP)

Document Type: Standard Operating Procedure (SOP)

Department: Patient Registration & Inpatient Services

Version: 1.0

Effective Date: January 1, 2026

Review Frequency: Annual

Page 1: Introduction and Purpose

1.1 Purpose

The purpose of this SOP is to establish standardized procedures for patient admission, transfer, discharge, and related documentation to ensure efficient patient flow, quality care, patient safety, and regulatory compliance.

1.2 Scope

This SOP applies to:

- Front Office Staff
- Registration Department
- Nursing Staff
- Physicians
- Billing Department
- Insurance Coordinators
- Medical Records Department
- Ward Administration

1.3 Objectives

- Ensure smooth patient admission.
 - Reduce administrative delays.
 - Improve patient experience.
 - Ensure accurate documentation.
 - Facilitate safe discharge planning.
-

Page 2: Definitions and Responsibilities

2.1 Definitions

Admission

The formal process of registering and assigning a patient to receive inpatient care.

Discharge

The official release of a patient from hospital care following treatment completion or physician approval.

Transfer

Movement of a patient from one department, ward, or facility to another.

Emergency Admission

Immediate admission due to urgent medical needs.

2.2 Responsibilities

Registration Staff

- Verify patient identity.
- Complete admission forms.
- Generate patient registration number.

Nursing Staff

- Assess patient condition.
- Record vital signs.
- Orient patient to ward.

Physicians

- Evaluate patient.
- Approve admission.
- Determine discharge readiness.

Billing Team

- Process payments.
- Verify insurance approvals.
- Prepare final bill.

Page 3: Admission Process Overview

3.1 Admission Sources

Patients may be admitted through:

1. Outpatient Department (OPD)
2. Emergency Department (ED)
3. Referral Hospitals
4. Direct Admissions
5. Surgical Admissions

3.2 Admission Workflow

Step 1: Physician recommends admission.

Step 2: Registration department creates admission record.

Step 3: Patient receives unique Admission Number.

Step 4: Bed allocation is initiated.

Step 5: Nursing assessment completed.

Step 6: Initial treatment plan established.

3.3 Admission Categories

Elective Admission

Planned admissions for scheduled treatments.

Emergency Admission

Urgent admissions requiring immediate care.

Day-Care Admission

Admission for procedures completed within the same day.

Page 4: Registration and Documentation Requirements

4.1 Required Documents

Patients must provide:

- Government-issued identification
- Address proof
- Insurance card (if applicable)
- Referral letter (if applicable)
- Previous medical records

4.2 Registration Information

The following information must be collected:

- Full Name
- Date of Birth
- Gender
- Contact Number
- Emergency Contact
- Address
- Insurance Details

4.3 Medical Record Creation

The Medical Records Department shall:

- Create patient file.
- Generate Patient ID.
- Assign admission number.
- Store documentation securely.

4.4 Consent Documentation

Required consent forms include:

- General treatment consent
 - Surgery consent (if applicable)
 - Blood transfusion consent
 - Special procedure consent
-

Page 5: Bed Allocation and Ward Admission

5.1 Bed Assignment Procedure

The Bed Management Team shall:

1. Verify bed availability.
2. Match bed category requested.
3. Allocate appropriate ward.

5.2 Ward Categories

- General Ward
- Semi-Private Room
- Private Room
- Deluxe Room
- Intensive Care Unit (ICU)

5.3 Nursing Admission Assessment

Within one hour of admission:

- Record vital signs.
- Obtain medical history.
- Assess allergies.
- Review medications.
- Evaluate fall risk.

5.4 Patient Orientation

Staff shall explain:

- Ward facilities
 - Visiting hours
 - Emergency procedures
 - Nurse call system
-

Page 6: Insurance Verification and Financial Clearance

6.1 Insurance Verification

Insurance desk shall verify:

- Policy validity
- Coverage eligibility
- Pre-authorization requirements
- Exclusions and limitations

6.2 Cashless Admission Process

Steps:

1. Submit insurance documents.
2. Obtain pre-authorization.
3. Receive insurer approval.
4. Complete admission.

6.3 Self-Pay Patients

Patients shall:

- Pay admission deposit.
- Sign financial responsibility agreement.

6.4 Financial Documentation

Required records:

- Deposit receipts
 - Insurance approvals
 - Billing estimates
 - Payment acknowledgments
-

Page 7: Inpatient Care Management

7.1 Daily Clinical Monitoring

Healthcare teams shall:

- Monitor patient condition.
- Update treatment plans.
- Record progress notes.

7.2 Physician Rounds

Physicians shall:

- Conduct daily evaluations.
- Review diagnostic reports.
- Modify treatment plans as needed.

7.3 Nursing Documentation

Nurses shall document:

- Vital signs
- Medication administration
- Clinical observations
- Patient responses

7.4 Interdepartmental Coordination

Coordination shall occur between:

- Laboratory
 - Pharmacy
 - Radiology
 - Rehabilitation Services
 - Billing Department
-

Page 8: Discharge Planning Process

8.1 Discharge Criteria

A patient may be discharged when:

- Treatment objectives are achieved.
- Clinical condition is stable.
- Physician approves discharge.

8.2 Early Discharge Planning

Planning begins within 24 hours of admission.

Considerations include:

- Home care needs
- Follow-up appointments
- Medication requirements
- Rehabilitation support

8.3 Patient Education

Before discharge, staff shall provide:

- Medication instructions
- Dietary guidance
- Activity recommendations
- Follow-up schedule

8.4 Discharge Checklist

Checklist includes:

- Physician approval
 - Nursing clearance
 - Pharmacy reconciliation
 - Billing clearance
 - Documentation completion
-

Page 9: Discharge Documentation and Billing

9.1 Required Discharge Documents

Patients shall receive:

- Discharge Summary
- Prescriptions
- Investigation Reports
- Follow-up Instructions
- Medical Certificates (if applicable)

9.2 Discharge Summary Contents

The summary shall include:

- Diagnosis
- Treatment provided
- Procedures performed
- Medications prescribed
- Follow-up recommendations

9.3 Billing Finalization

Billing Department shall:

- Prepare final bill.
- Apply insurance adjustments.
- Collect remaining balance.
- Generate payment receipt.

9.4 Medical Record Closure

Medical Records Department shall:

- Archive patient file.
 - Update electronic records.
 - Ensure document completeness.
-

Page 10: Special Situations and Quality Assurance

10.1 Discharge Against Medical Advice (AMA)

Patients leaving AMA shall:

- Receive counseling.
- Sign AMA documentation.
- Acknowledge risks.

10.2 Patient Transfer Procedure

Transfers require:

- Physician authorization.
- Transfer documentation.
- Receiving unit acceptance.

10.3 Deceased Patient Procedure

The hospital shall follow:

- Legal reporting requirements.
- Death certification process.
- Family notification procedures.

10.4 Quality Monitoring Indicators

The hospital shall monitor:

- Admission turnaround time
- Bed occupancy rate
- Discharge processing time
- Patient satisfaction
- Readmission rates

10.5 SOP Review

This SOP shall be reviewed annually or whenever operational changes occur.

Document Control

Document Name: Admission & Discharge Standard Operating Procedure

Hospital: CareSync Hospital

Version: 1.0

Department Owner: Patient Registration & Inpatient Services

Approved By: Medical Superintendent

Review Cycle: Annual

Effective Date: January 1, 2026

Classification: Internal Operational Document

End of Document